

D12 — Medicine Attending Note

Dr. M. Torres, Medicine Attending | Medicine | 02/22/2026 08:00

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

MEDICINE ATTENDING PROGRESS NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/22/2026 08:00 | HD #5 | Provider: Dr. M. Torres, Medicine Attending

Discussed case with GI (Dr. Nakamura) and IR (Dr. Alvarez). Both have provided thoughtful recommendations.

DECISION RE: GI BLEED MANAGEMENT:
Given that primary endoscopic hemostasis with clips failed within 24 hours, I am electing to proceed with IR embolization rather than repeat EGD. My rationale:

1. Early clip failure suggests the lesion characteristics (vessel size, ulcer depth) are not favorable for endoscopic hemostasis alone
2. The patient is hemodynamically unstable and ongoing blood loss needs definitive management
3. IR embolization offers >90% technical success vs 70-80% for repeat endoscopy
4. Risk of perforation with repeat cautery in a patient on systemic steroids is a valid concern

IR scheduled for this afternoon.

Anticoagulation timeline: will follow IR recommendation of minimum 5 days post-embolization. Earliest restart 02/27/2026. Cardiology to advise on optimal timing.

Prednisone: continuing at 40 mg (Day 3 of oral taper). Cannot discontinue given adrenal suppression risk. PPI coverage essential.

Will transfer to MICU following IR procedure for hemodynamic monitoring.

Electronically signed by: Dr. M. Torres, MD
Date: 02/22/2026 08:30